



Health & Social Services Department

Donald R. Rowe, Director

Public Health - Mental Health - Public Guardian
Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

Public Health Division
Thomas L. Charron, M.D., M.P.H.
Health Officer
Deputy Director Health and Social Services

EMERGENCY MEDICAL SERVICES
717 Missouri Street, Suite B
Fairfield, CA 94533
(707) 421-6685
FAX (707) 421-6682

EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #6165

EFFECTIVE DATE: 06/01/93

APPROVED BY:

THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

ALLEN MORINI, D.O., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: DONOR CARD SEARCH BY EMS PERSONNEL

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, 7152.5(b)

PURPOSE/POLICY:

Section 7152.5 of the CA Health & Safety Code requires EMS personnel to make a "reasonable search" for a document, or other information, identifying the bearer as a donor or as an individual who has declined to make an anatomical gift under the Uniform Anatomical Gift Act (Section 7150, CA H&S Code).

PROCEDURE

In general, EMS personnel should not search a patient's personal belongings to locate a donor card. When it appears that an individual may be a donor, a "reasonable search" consists of asking a family member (if present) about the possibility of a donor card and notifying of the results of the search:

- The receiving hospital, if transported, **or**
- A law enforcement office, if not transported.

DOCUMENTATION

If personal belongings are searched, the search **must** be done in the presence of a witness, and the name of the witness, details regarding the search, what was found and who was notified shall be documented on the PCR. EMS will leave the "patient medical record copy" with the body for proper notification of the Coroner.

Documentation that the **search was conducted** shall be entered onto the PCR in the special scene conditions section utilizing the number code 31 (see example below). Documentation that the **search was conducted and a donor card was found** (regardless of the information included therein) shall be entered onto the PCR in the special scene conditions section utilizing the number code 32 (see example below).

SPECIAL SCENE CONDITIONS	
☐	ALS W/O BHC
☐	HAZ-MAT
☐	EMS Aircraft
☐	Major Trauma Patient
☐	Transfer Patient
☐	OTHER <u>31</u> or
☐	OTHER <u>32</u>

If a document of gift or evidence that the patient declined to assign an anatomical gift is located by search **and** the patient is transported to a hospital, the receiving hospital shall be notified of the contents of the document.

If the individual is determined to be dead by the Criteria for Non-Resuscitation Policy or Discontinuation of CPR Policy, EMS personnel shall notify a law enforcement officer to perform the search.

Solano County Health & Social Services Department

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Public Health Services
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Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Patrick O. Duterte, Director

Steven P. Whiteley, MD
EMS Agency Medical Director

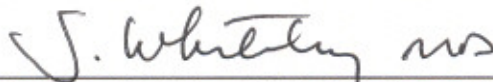
EMERGENCY MEDICAL SERVICES AGENCY
275 Beck Avenue MS 5-240
Fairfield, Ca. 94533
(707) 784-8155 FAX (707) 421-6682
www.solanocounty.com

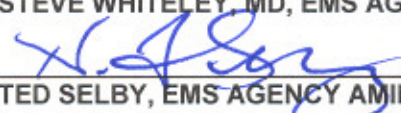
Ted Selby
EMS Agency Administrator

POLICY MEMORANDUM 6170

DATE: August 31, 2011

REVIEWED/APPROVED BY:


STEVE WHITELEY, MD, EMS AGENCY MEDICAL DIRECTOR


TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: INFECTION CONTROL/COMMUNICABLE DISEASE

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, SECTIONS 1797.188 & 1797.189; RYAN WHITE COMPREHENSIVE AIDS RESOURCES EMERGENCY CARE ACT (PUBLIC LAW 101-3810), Chapter 1.155 (commencing with Section 199-65) to Part 1 of the Health & Safety Code.

PURPOSE/POLICY:

Preventing exposure to Infectious Disease, preventing the spread of Infectious Disease and reporting an exposure to Infectious Disease is EVERY EMS Worker's responsibility. This Policy deals with five (5) components:

- | | |
|--------------------------|--|
| I. Universal Precautions | IV. Medical Facility Reporting Obligations |
| II. Equipment Sanitation | V. Prevention |
| III. Exposure Reporting | |

DEFINITION:

Exposure is defined as contact with blood or body fluids to:

- I. Non-intact skin or mucus membranes; and/or
- II. Accidental puncture/cut of skin or mucus membranes; and/or
- III. Full facial contact to an aerosolized moistened particulate matter (mist, cough).

I. UNIVERSAL PRECAUTIONS

The Centers for Disease Control and Prevention (CDC) recommends universal precautions emphasizing that **all patient contacts should be assumed to be infectious for blood borne and/or body fluid pathogens; and may be infectious with aerosolized pathogens.**

Universal Precautions **shall** include:

- A. Appropriate latex gloves (put on prior to patient contact) for touching blood and body fluids, changed after each contact.
- B. Appropriate masks, protective eye wear, gowns, or aprons should be worn during procedures that could generate splashes of body fluid.
- C. Hand washing with soap and water and/or disinfectant solution immediately after contact with blood or body fluid, and after removing gloves.
- D. Extra precautions to be used to prevent injuries generated by “sharps.” Proper disposal of used needles without recapping them in a puncture resistant container is mandated, as well as appropriate bio-hazard disposal.
- E. Pocket masks or other appropriate ventilation devices as indicated.
- F. EMS personnel who have open sores or dermatitis should refrain from direct patient contact and sanitation of potentially infected equipment until the condition is resolved.

II. EQUIPMENT SANITATION

The sanitation of EMS equipment is a vital link in preventing the spread of any infectious disease. Proper disposal of all non-reusable equipment shall follow Manufacturer Guidelines.

- A. **Laryngoscope blades, McGill Forceps, and Endotracheal Tube Stylets** shall **ALL** be cleaned with soap and water and then disinfected with the high level disinfectant chemical sterilant, Glutaraldehyde, as recommended by APIC (Association for Practitioners in Infection Control) guidelines for Infection Control Practice, **OR** solution/mechanism approved by the County Health Officer.
- B. **General clean-up** of the Ambulance compartment shall follow both CDC guidelines and Cal OSHA Standards.

III. EXPOSURE REPORTING

Section 1797.188 of the Health and Safety Code requires hospitals to notify the County Health Officer when EMS personnel have been exposed to an infectious disease. The Ryan White Act requires medical facilities to notify the designated officer when emergency response employees are exposed to specified infectious diseases during the transport of a patient to the hospital.

The law further states that it is the responsibility of EMS personnel to provide information to the receiving hospital when they perceive that a significant exposure has occurred, (e.g., oral contact to blood or body fluids, including but not limited to mouth-to-mouth resuscitation; blood splash in the face; or accidental needle stick.)

A. Transported Cases

All EMS Personnel, including Police and Fire Department Personnel, should report any significant exposure to the hospital where the patient was transported. Reporting forms are available in all of the Hospital Emergency Departments (see attachment A). The form should be turned into the Base Hospital Liaison Nurse, Charge/Lead Nurse, or the Infectious Disease Nurse and/or designee at the receiving hospital. The exposed individual shall keep a copy of the exposure report for their record. Significant exposures to positive infectious disease sources will be followed-up on by each hospital and reported to the “designated officer” of the agency involved. Individual calls from emergency response personnel will not be answered.

B. Non-Transported Cases

Exposure cases involving a non-transported victim (Coroner’s case, Against Medical Advice [AMA], etc., must be reported on an Exposure Form [Attachment A]). Follow-up will occur through the Solano County Public Health Communicable Disease Program coordinating Infectious Disease reporting for Solano County. Forms should be mailed or faxed to:

Solano County Public Health Communicable Disease Program
355 Tuolumne Street
Vallejo, CA 94590
FAX NUMBER: 707-553-5649.

A telephone report should be made in addition by calling 707-553-5555, Monday through Friday, during normal business hours.

C. Out-of-County Transfers

Out-of- County transfers where an exposure has occurred shall be reported on an exposure form to the patient's final destination hospital.

In all cases, the responsibility for reporting Infectious Disease exposures belongs to the exposed EMS worker. This report must be made to both the hospital and employer.

IV. MEDICAL FACILITY REPORTING OBLIGATIONS

- A. **Facility-Initiated Reports to Designated Officer.** If a patient is determined to have an **airborne infectious disease**, the facility must report this to the designated officer of the emergency response employees who transported the patient to the facility.
1. The only currently disease specified airborne in this category is **infectious pulmonary tuberculosis**. Persons undergoing tuberculosis drug therapy, and skin test converters who are not infectious, should not be reported.
 2. The facility must notify only the designated officer of the emergency response employees who transported the victim, not those who may have also treated the victim prior to transport. Any emergency response employee who attended, treated, assisted, or transported a patient may request a determination as to whether there was an exposure to an infectious disease (Refer to Section B).
 3. This notification must be made as soon as it is practicable, but not later than 48 hours after determination is made.
 - a. The facility must inform the designated officer of the name of the infectious disease involved (infectious pulmonary tuberculosis), and the date on which the patient was transported by emergency response employees to the facility. No other information, including the name of the patient, should be released.
 - b. If the required notification to the designated officer is made by mail, the facility must inform the (by telephone or in person) that the notification has been sent; the designated officer must, within 10 days, inform the facility whether the notification was received.
 4. If a patient dies at or before reaching the hospital, the medical facility ascertaining the cause of death (if not the hospital) must make the required notification.

- B. Facility Response to designated office inquiries. If any emergency response employee believes he or she may have been exposed to an infectious disease from a patient transported to a medical facility, the emergency response employees may request his or her employer to review the circumstances surrounding the potential exposure.
1. Infectious diseases addressed in policy are:
 - a. Airborne Diseases which include, but are not limited to:
 - Infectious Pulmonary Tuberculosis
 - b. Other Infectious Diseases which include, but are not limited to:
 - Hepatitis B
 - HIV infection (including AIDS)
 - Diphtheria (*Corynebacterium diphtheriae*)
 - Meningococcal Disease (*Neisseria Meningitidis*)
 - Plague (*Yersinia Pestis*)
 - Hemorrhagic fevers (Lassa, Marburg, Ebola, Crimean-Congo, and other viruses yet to be identified)
 - Rabies
 2. If the designated officer determines that the emergency response employees may have been exposed to one or more of these infectious diseases, the designated officer must submit a request for response to the facility to which the patient was transported. This request must be in writing, contain a statement of the facts surrounding the circumstances of the potential exposure, and be signed by the designated officer.
 3. The facility must evaluate the facts submitted by the designated officer and determine if the emergency response employee was exposed to an infectious disease listed above based on the medical information possessed by the facility regarding the patient.
 4. If the facility determines that an emergency response employee was exposed to an infectious disease listed above, the facility must notify the designated officer in writing as soon as is practicable, but no later than 48 hours after receiving the request.
 - a. The facility must inform the designated officer of the name of the infectious disease involved and the date on which the patient was transported by emergency response employees to the facility. No other information, including the name of the patient, should be released.
 - b. If the required notification to the designated officer is made by mail, the facility must inform the designated officer (by telephone or in person) that the notification has been sent; the designated officer must, within 10 days, inform the facility whether the notification was received.

5. If the facility determines that an emergency response employee was not exposed to a listed infectious disease, the facility must notify the designated officer in writing as soon as practicable, but not later than 48 hours after receiving the request.
6. If a facility finds that the facts are insufficient to determine whether the emergency response employee was exposed to a listed infectious disease, the facility must notify the designated officer in writing of the insufficiency of the facts as soon as practicable, but not later than 48 hours after receiving the request.
 - a. If the designated officer requests, the County Health Officer may evaluate the request and the response, and make an independent determination and response to the designated officer.
 - b. If the County Health Officer believes the facts provided to the facility were sufficient or the County Health Officer obtains additional and sufficient facts, the County Health Officer will submit the request to the facility. The facility must then provide the designated officer the appropriate response.
7. If a facility determines that it possesses no information on whether the patient had an infectious disease listed above, the facility must notify the designated officer in writing of the insufficiency of the medical information as soon as is practicable, but no later than 48 hours after receiving the request. However, if the facility later determines that the patient had a listed infectious disease, the facility must notify the designated officer (See Section C below for specific requirements).
8. If a patient dies at or before reaching the hospital, and the hospital receives a request for response under the Act, the hospital must provide a copy of the request to the facility ascertaining the cause of death, if there is a different facility involved. That medical facility then has the responsibility of making the required determination and appropriate response.

C. Limitations on Facility Duties

The Ryan White Comprehensive Aids Resources Emergency Care Act specifically limits the responsibilities of the facility as follows:

1. The duties of facilities under the Ryan White Comprehensive Aids Resources Emergency Care Act terminate at the end of the period during which the facility provides medical care to the victim for conditions arising from the emergency, or at the end of the 60-day period beginning on the date the victim is transported by emergency response employees to the facility, *whichever period is shorter* – except that the facility's duties will continue if a request is received within 30 days of the date of the applicable period.

EXAMPLES:

- Example 1: If a victim is transported to a medical facility and released after two days, the facility must respond to a request if it is received within 30 days of the date the victim was discharged from the facility.
- Example 2: If the victim remains in the facility for more than 60 days, the facility must respond to a request if it is received within 30 days of the expiration of the 60-day period (this turns out to be 90 days from the date of the alleged exposure, which would be the maximum period of time that the facility would have notification obligations for that exposure).
2. The Ryan White Comprehensive Aids Resources Emergency Care Act does not authorize or require a facility to test any patient for any infectious disease. As a result, facilities and practitioners must follow existing consent requirements, including the requirements for informed and written consent for HIV tests. In this regard, testing may fall under Federal and State regulations, State - 8 CA CCR § 5193(f)(3), Federal - 29 C.F.R. §1910.1030(f)(3), requiring employers to test a source individual's blood for HIV and Hepatitis B following any occupational exposure - but only after consent is obtained from the source individual. Under some circumstances, if the source patient or the authorized legal representative of the source patient refuses to an HIV test after a documented effort has been made to obtain consent, then any available blood or patient sample of the source patient may be tested. A new sample may not be obtained. The source patient or authorized legal representative of the source patient shall be informed that an available blood sample or other tissue or material will be tested despite his or her refusal, and that the exposed individual shall be informed of the HIV test results. If a source patient is incapacitated and therefore is unable to provide informed consent and has no legal representative, then HIV testing on the source patient or available blood or tissue of the source patient shall not be permitted. (Refer to Chapter 1.155 commencing with Section 199.65 of Part 1 of Division 1 of the Health and Safety Code.)
 3. The Ryan White Comprehensive Aids Resources Emergency Care Act also does not authorize or require any facility, designated officer, or emergency response employee to disclose identifying information with respect to a patient or an emergency response employee.
 4. The Ryan White Comprehensive Aids Resources Emergency Care Act does not authorize any emergency response employee to fail to respond to, or deny services to, any victim of an emergency.

V. COMMUNICABLE DISEASE EXPOSURE PREVENTION

- A. All Solano County EMS Service Providers shall develop an Infection Control Plan consistent with Cal OSHA (Occupational Safety and Health Act including Aerosol Transmissible Disease (ATD) Standard) and CDC requirements; to be approved by the EMS Agency.
- B. Quality Assurance mechanisms for compliance addressing all five components of this EMS Policy shall be included as part of the Infection Control Plan.
- C. Infection Control Plans shall include an immunization program, at no cost to the employee, of MMR (Measles, Mumps & Rubella), HBV (Hepatitis B), annual tuberculosis PPD (Purified Protein Derivative), and/or any other immunizations specified by the County Health Officer.

Solano County Health & Social Services Department

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Gerald Huber, Director

Aaron E. Bair, MD, MS
EMS Agency Medical Director

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275 Beck Avenue MS 5-240
Fairfield, Ca. 94533
(707) 784-8155 FAX (707) 421-6682
www.solanocounty.com

Terri Gill, MA
EMS Agency Administrator

Date: June 8, 2015

POLICY 6170 - ATTACHMENT A

Request for Communicable Disease Follow-Up for Possible Exposure of Emergency Personnel
and/or First Responders.

Submission Date: _____ Receiving Hospital: _____

Name: _____ Agency Name: _____

Agency Address: _____

Agency Phone Number: _____ Fax Number: _____

Date of Exposure: _____ Incident Dispatch and/or PCR #: _____

Type of Exposure (Check all that apply):

- ☐ Blood and/or Body Fluids to non-intact skin or mucous membranes
☐ Accidental puncture/cut of skin or mucous membranes.
☐ Full facial contact to an aerosolized mist (cough)
☐ Other (Describe): _____

PATIENT NAME: _____ D.O.B. _____

MEDICAL RECORD #: _____

Agency Designated Officer (DO) Name _____ Signature _____

RECEIVING HOSPITAL FOLLOW-UP

Date received: _____

- ☐ No reportable active infectious disease was identified. Medical follow-up is unnecessary.
☐ An active reportable infectious disease was identified. Follow-up is indicated. Follow
your agency's infection control policy.

Infectious disease identified: _____

Date patient transported: _____

Signature: _____ Date: _____

Designated Officer reported to: _____ Date: _____

Method: _____

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Ted Selby
Emergency Services Bureau Chief

POLICY 6170 - INFECTION CONTROL OFFICERS

Effective: 6/26/2013

Note: If "area code" not noted, it is "707".

Organization	Contact	Phone	Fax#
Benicia Fire Dept.	K.C. Smith, Acting Div. Chief	746-4275	745-4425
Benicia Police Dept.	Sgt. John Daley	746-4248	746-0131
Calstar	Scott Wallace RN	925-798-1666	925-798-5484
CHP	Cpt. Susan Ward	428-2100	428-2039
CHP-H-30	Sgt. Greg Crippen	257-0103	257-2756
Cordelia Fire	Joseph Dodson	864-0468	864-8607
Dixon Fire	Aaron McAlister, Asst. Chief	678-7060	678-4251
Fairfield Fire Dept.	Anthony Velasquez	436-7226	399-0862
Fairfield Police Dept.	Sgt. Chad Tigert	428-7360	428-7427
Kaiser Hospital Vallejo	Steve Parodi, MD	651-4697	651-2783
	Carleen Lawrence, RN	651-2107	651-2108
Medic Ambulance	Matt Copenhaver	644-1771, ext 15	644-1784
Montezuma Fire Dept./Ryer Island FD	Joe Rosewall, Chief	374-5962	374-4999
NorthBay Health Care System	Daman Mott, R.N.	455-2154	
OEA Aerospace	Linda Marr	399-1895	422-3242
Reach	Sean Russell		
	Michelle Glancey (Dir/HR)	974-1250	575-8049
Rio Vista Fire	Max Etchieson	374-2233	374-6324
Six Flags Marine Wld.	Rhonda Dutton	556-5362	556-5272
Solano Sheriff Dept.		421-6330/	
	Sheriff Dispatch/Bob Powell	580-4980-cell	421-6383
Suisun City Fire Dept.	Mike O'Brien, Chief	425-9133	421-7398
Suisun Fire Dist.	Frank Cereda, Asst. Chief	425-3605	425-3615
Suisun Police Dept.	Anthony Welch	421-7383	421-7273
Sutter Solano Med. Ct.	Amal Johnson	554-5364	554-5082
Travis AFB DGMC	Capt. Dailey	423-7922	
Travis AFB Fire	Glen Munn	424-6771	
US CST Grd Carquinez	Justin D. Eaton	643-2975	643-5737
US CST GRD Rio Vista	Robert Goley	374-2871	374-6478
Vacaville Fire Depart.		449-6116/	
	Butch Lee, Fire Capt.	432-7265-pgr	449-1844
Vacaville Fire Dist.	Howard Wood, Chief	447-2252	447-2769
Vacaville Police Dept.	Lt. Craig Courtemanche	689-3734	449-5232
Valero Refinery		745-7571/	
	Linda Moore RN	745-7562	745-7903
Vallejo Fire Dept.	Danny Richards RN	648-5302	553-7273
Vallejo Police Dept.	Lori Lee	648-5291	648-4490

* PLEASE FORWARD ANY CHANGES TO SOLANO COUNTY EMS (707-784-8155).

GERALD HUBER
Director

DEPARTMENT OF HEALTH & SOCIAL SERVICES

Medical Services Division



**SOLANO
COUNTY**

BRYN MUMMA, MD, MAS
EMS Agency Medical Director

TED SELBY
EMS Agency Administrator

(707) 784-8155
www.solanocounty.com

EMERGENCY SERVICES BUREAU

355 Tuolumne Street,
Suite 2400, MS 20-240
Vallejo, CA 94590

POLICY MEMORANDUM 6180

Implementation Date: December 1, 2013

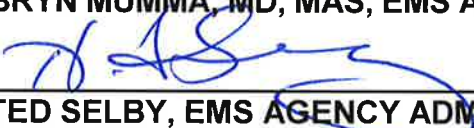
Revised Date: September 15, 2018

Review Date: September 15, 2020

REVIEWED/APPROVED BY:

 MD, MAS

BRYN MUMMA, MD, MAS, EMS AGENCY MEDICAL DIRECTOR



TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: MULTI CASUALTY INCIDENT (MCI) RESPONSE

AUTHORITY: California Health & Safety Code, Division 2.5, Chapter 3, Article 4, Sections 1797.151 and 1797.153; Chapter 4, Article 1, Section 1797.220; Chapter 5, Sections 1798(a) – (b) and 1798.2; Chapter 6, Article 3, Section 1798.170
California Code of Regulations, Title 22, Division 9, chapter 4, Article 2, Sections 100145, 100146, and 100148

PURPOSE:

- A. Establish common framework for single or multiple agencies to follow when responding to a Multi Casualty Incidents (MCI).
- B. Documentation Protocols associated with requesting mutual aid or assistance for MCIs.
- C. Outline standards to ensure Command and Control, Notification, Communication, and Patient Distribution and Tracking are managed consistently throughout the County.

I. DEFINITIONS:

- A. MCI – An incident with a significant number of patients/victims requiring immediate care and treatment which overwhelms the current Emergency Medical Services (EMS) system. This should be not less than five patients.
- B. START Triage – The system that field personnel will use to triage (sort) patients/victims into four categories: red (immediate), yellow (delayed), green (walking wounded), and black (dead).
- C. ReddiNet – An internet-based application that Solano County EMS uses to initiate hospital bed capacity polls for the various injured patients/victims at the scene of a MCI, distribute patients to specific hospitals, and assist with patient/victim and family re-unification. This application is also available as an app for mobile devices.
- D. MCI Base Hospital – A Solano County designated Base Hospital, employing Mobile Intensive Care Nurses (MICN), that is responsible for dispersal of patients during declared MCIs. (Patient destination may be to receiving hospitals in and out of county.)
 - 1. The MCI Base Hospital for Solano County shall be the designated Level II Trauma Center.
 - 2. MICNs will be authorized per Solano County EMS Policy 3500, Roles and Authorization/Reauthorization Requirements for MICNs

II. MCI Personnel Procedure/Guidelines:

- A. An incident will generate a response from an appropriate Public Safety Answering Point (PSAP).
- B. After an initial evaluation of the scene, the appropriate command staff will institute the Incident Command System (ICS) structure, declare a MCI if appropriate, and designate the appropriate level. An initial request for medical mutual aid can be made if necessary.
 - 1. MCI Level Definitions:
 - a. Level I – Expand Medical Event
Five to fifteen (5-15) patients
 - b. Level II – Major Medical Event
Sixteen to fifty (16-50) patients
 - c. Level III – Medical Disaster
More than fifty (50) patients

2. Solano County Base Hospitals and Trauma Centers agree to accommodate the following numbers and severities of patients at the initial activation of an MCI:
 - a. Traumatic MCI
 - i. Trauma Centers – 2 red, 5 yellow, and 5 green
 - ii. Non-trauma Centers – 5 green
 - b. Medical MCI
 - i. All Base Hospitals – 2 red, 5, yellow, and 5 green
3. Medical Mutual Aid:
 - a. Using the Medical Mutual Aid Matrix (Attachment A), determine the geographic location of the incident and request through Solano Dispatch the appropriate level of response. Report the projected number of injured requiring transport and their triage categories.
 - i. When activating the Medical Mutual Aid Matrix ensure that the Exclusive Operating Area (EOA) Ambulance Contractor has been consulted and is a part of the ICS Command Structure for decision making.
 - ii. Solano County Dispatch will also notify the Medical Health Operational Area Coordinator (MHOAC) for assistance when requesting mutual aid especially when out-of-county resources are required.
 - iii. When out-of-county resources or supplies are needed, the MHOAC should contact the Regional Disaster Medical Health Specialist to facilitate a regional response.
 - b. Mutual Aid/Assistance Agreements between private ambulance providers may be entered into between operators, agencies, and/or Local Emergency Medical Services Agencies (LEMSAs).
 - c. Publicly operated ambulances must adhere to the State of California Master Mutual Aid Agreement.
- C. The Incident Commander (IC), or designee, will delegate contacting the Designated MCI Base Hospital and provide notification of the declared MCI, location, and estimated number of casualties and their categories, e.g. red, yellow, and green to the appropriate staff member following ICS standards.
 1. The Designated MCI Base Hospital will use ReddiNet, following Policy 7106, and make appropriate system hospital notifications.
 2. A Solano County Authorized Mobile Intensive Care Nurse (MICN) will be the point of contact at the MCI Base Hospital and will delegate transport destination to the IC or designee.
- D. Initial Field Crews must begin triaging patients using a triage system with the approved Triage Tags, e.g. START

- E. The IC will fill the appropriate ICS positions to ensure the event and patients are managed safely. For the medical response these position(s) may include, but not be limited to: Medical Group Supervisor, Triage Unit Leader, Treatment Unit Leader, and Transportation Group Supervisor.
1. The Medical Group Supervisor should consider utilizing all Solano County Resources, e.g. the Disaster Medical Support Unit (DMSU).
 - a. The Medical Group Supervisor (MGS) should contact the MCI Base MICN for transport destination direction to system hospitals.
 - i. Communication(s) with hospitals should be via cellular phone or the MedNet Radio.
 2. The Transportation Group Supervisor should consider using alternate forms of transportation for patients, e.g. city buses, trucks, and helicopters, if necessary.
 - a. The Transportation Group Supervisor should coordinate with the MCI Base MICN to obtain transport destination and notify when a transport unit is en route to a receiving facility.
 - b. The MCI Base MICN will access ReddiNet to assign and update transport destination for all other receiving facilities.
 - i. Field crews will utilize Attachments B, C, D, and E to track all transport movements for late documentation.
 - ii. Communication(s) with hospitals should be made via cellular phone or MedNet radio.
 3. The Treatment Unit Leader should determine how to maximize treatment supplies. Depending on the size of the event this may include utilizing treatment supplies from local hospitals, pharmacies, or clinics.
 - a. All requests for outside assistance for equipment shall be requested through the Medical Health Operational Area Coordinator (MHOAC).
- F. Prehospital staff/Transporting staff at the scene of the MCI:
1. Patients will be transported based on the transportation Group Supervisor's information.
 2. When encountering individual(s) that are not injured, Policy 6125, Refusal of Medical Assistance (RMA), may be used to allow those individual(s) to leave the scene. In Lieu of signing the RMA form, individual(s) may sign the triage tag. Demographic information must be captured on all patients leaving the scene of a MCI on the appropriate form submitted to the EMS Agency.
 - a. Uninjured individuals wanting to leave the scene should be advised that they may seek medical attention at any time.
- G. The MGS should notify the MCI Base MICN once the last patient has been transported so ReddiNet can be updated.
- H. Non-injured individuals will not be transported to hospitals.

- I. All hospitals will follow the procedures outlined in Policy 7106, Use of ReddiNet during an MCI, when receiving, treating, and discharging MCI patients.

III. MCI Quality Improvement:

Review of MCIs will be based on the MCI Level. **All** MCIs will have the following quality improvement documentation submitted to the EMS Agency.

A. Level I MCI Review:

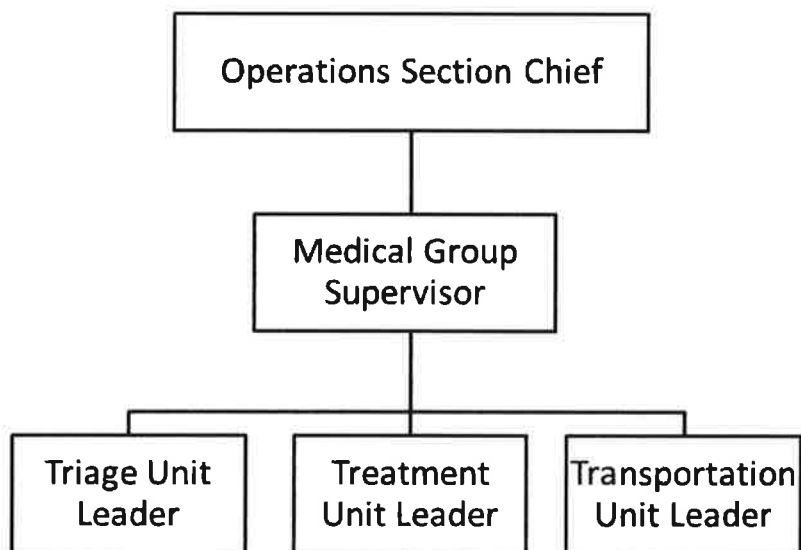
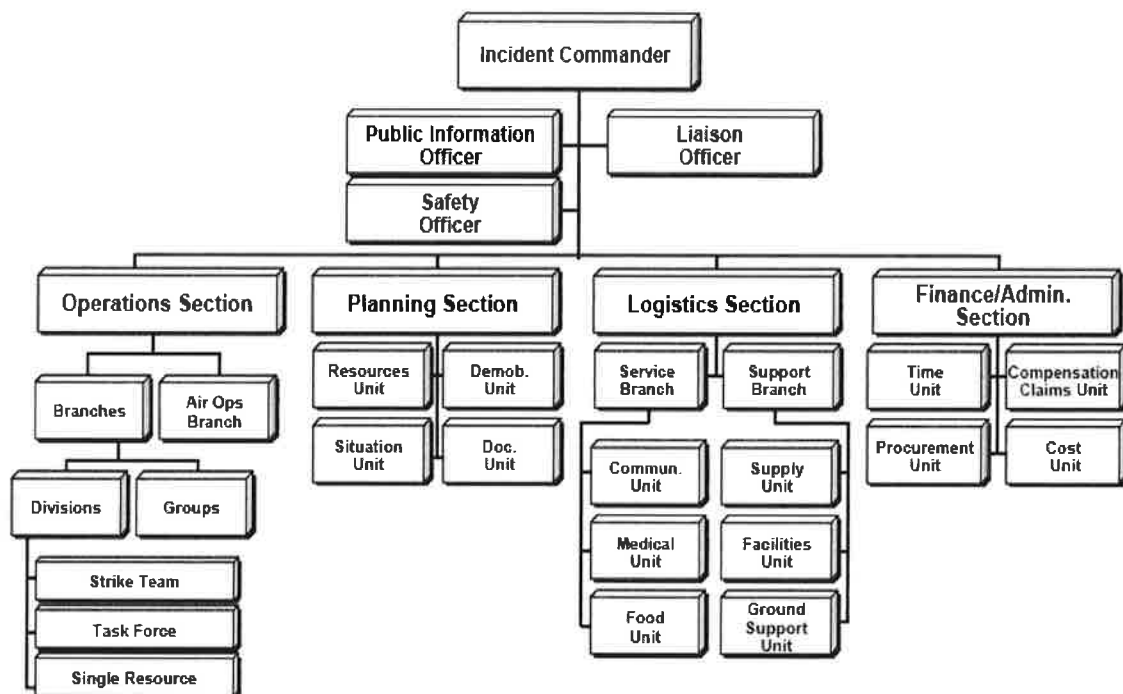
1. A Medical Group Supervisor Summary Report (Attachment F) will be completed and submitted to the EMS Agency within 48 hours of resolution of the incident.
2. After review of the above information the EMS Agency may meet with the involved organizations to review and discuss the incident. The IC and command staff should be present.

B. Level II and III MCI Review:

1. A Medical Group Supervisor Summary Report (Attachment F) and a Field Advisory Report (FAR) from the IC will be completed and submitted to the EMS Agency within 48 hours of resolution of the incident. The FAR should contain a brief review of the incident and the strengths and areas for improvement.
2. A formal, review/critique of the incident should be scheduled within 72 hours of the resolution of the incident.
3. The IC, Emergency Operations Center (EOC) Director, or designee shall schedule and conduct the review/critique.
4. All entities involved in the resolution of, or response to, the incident shall be invited and encouraged to participate, both in discussion at the time and in formal written reviews of the events. Other interested organizations and surrounding LEMSA representatives will also be invited to attend.
5. The lead agency where the MCI occurred will issue a final After Action Report (AAR) containing any written reports, discussions, conclusions, and recommendations for handling future incidents within 60 days of the review being conducted.
6. All entities involved with the incident will also perform a "lessons learned" presentation at the next scheduled Process Improvement meeting.

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Policy 6180 MCI – ICS Organizational Chart



Note: This organizational chart is a representation of what ICS may be. Every incident is different and may not require all the positions shown. ICS positions may be added or subtracted as the incident dictates.

Policy 6180 Attachment A – Mutual Aid Ambulance Response Matrix

Based on incident general geographic location

Assumption: A large scale Multi-Casualty Incident (MCI) in specific geographic area requiring multiple ambulance response. This response matrix should be used when the number of non-ambulatory critical patients exceeds 30. In order to facilitate the transport of critical patients the <u>closest</u> Solano County transport resources must be used prior to requesting out of county transport resources. For non-critical ambulatory patients the use of local public transportation should be considered.		
Incident Location	Responder	
Vacaville Area, including Vacaville Fire Protection District area	a. Contact Vacaville Fire b. Contact Medic Ambulance c. Contact Travis Air Force Base i. Ambulances ii. Am-buses d. Contact Solano Dispatch i. Yolo County transport resource ii. Sacramento County transport resource (depending on location) iii. Napa County transport resource (depending on location) e. Consider using alternate transportation resources, e.g. city buses	
Dixon Area, including Dixon Fire Protection District area	a. Contact Medic Ambulance b. Contact Vacaville Fire c. Contact Travis Air Force Base i. Ambulances ii. Am-buses d. Contact Solano Dispatch i. Yolo county transport resource (depending on location) ii. Sacramento County transport resource (depending on location) e. Consider using alternate transportation resources, e.g. city buses	

Policy 6180 Attachment A – Mutual Aid Ambulance Response Matrix

Based on incident general geographic location

Incident Location	Responder
Fairfield/Suisun/Cordelia Area, including Suisun Fire Protection District area	a. Contact Medic Ambulance b. Contact Vacaville Fire c. Contact Travis Air Force Base i. Ambulances ii. Am-buses d. Contact Solano Dispatch i. Napa County transport resource (depending on location) ii. San Joaquin County transport resource (depending on location) iii. Yolo County transport resource (depending on location) iv. Contra Costa County transport resource (depending on location) e. Consider using alternate transportation resources, e.g. city buses
Rio Vista Area, Including Montezuma Fire Protection District Area	a. Contact Medic Ambulance b. Contact Vacaville Fire c. Contact Travis Air Force Base i. Ambulances ii. Am-buses d. Contact Solano Dispatch i. Sacramento County transport resource (depending on location) ii. Contra Costa County transport resource (depending on location) iii. San Joaquin County transport resource (depending on location) e. Consider using alternate transportation resources, e.g. city buses
Benicia Area	a. Contact Medic Ambulance b. Contact Vacaville Fire c. Contact Travis Air Force Base i. Ambulances ii. Am-buses d. Contact Solano Dispatch i. Contra Costa County transport resource (depending on location) ii. Napa County transport resource (depending on location) e. Consider using alternate transportation resources, e.g. city buses

Policy 6180 Attachment A – Mutual Aid Ambulance Response Matrix

Based on incident general geographic location

Incident Location	Responder
Vallejo Area	a. Contact Medic Ambulance b. Contact Vacaville Fire c. Contact Travis Air Force Base i. Ambulances ii. Am-buses d. Contact Solano Dispatch i. Napa County transport resource (depending on location) ii. Contra Costa County transport resource (depending on location) e. Consider using alternate transportation resources, e.g. city buses

Policy 6180 Attachment C – Hospital Resource Availability Sheet

[illegible]

Policy 6180 Attachment D – Casualty Recorder Worksheet

(I) Immediate/Red (D) Delayed/Yellow (M) Minor Green

[illegible]

Policy 6180 Attachment E – Ambulance Staging Resources Status Form

[illegible]

Policy 6180 Attachment F – Mass Casualty Incident Medical Group Supervisor Summary/Evaluation Report

Initial MCI Demographic Information:

1. Incident Number, Date, and Time: _____
2. Incident Location: _____
3. Incident Commander: _____
4. Level of MCI Declared: _____
5. Medical Group Supervisor: _____
6. Time Call Received: _____
7. Public Safety Arrival Time: _____
8. Public Safety Agency: _____
9. Time MCI Declared: _____
10. Base Hospital: _____
11. Time Solano Dispatch Notified: _____

EMS MCI Response:

1. Estimated Number of Initial Victims: _____
2. Estimated Number of Victim's Triage Categories: I - _____; D - _____; M - _____.
3. Number and type of transport resources requested: _____
4. Incident Command System (ICS) Utilized YES/NO; *if no please use a separate piece of paper to explain* _____
5. Initial Medical Group Supervisor: _____
6. Final Medical Group Supervisor: _____
7. Other Incident Command positions filled, if applicable, Provide Name and Agency:
 - a. Triage Unit Leader - _____
 - b. Treatment Unit Leader - _____
 - c. Transportation Group Supervisor - _____
 - d. Medical Supply Leader - _____
 - e. Transportation Staging Manager - _____
 - f. Helispot Manager - _____

Post MCI Evaluation:

Please explain "NO" answers on a separate page. Information will be used strictly for system improvement.

1. IC and/or Command Post easily identified by incoming ambulance personnel?
☐ Yes ☐ No
2. Adequate ambulance staging access, egress and area?
☐ Yes ☐ No
3. Ambulance personnel briefed upon arrival?
☐ Yes ☐ No
4. Patients triaged prior to movement?
☐ Yes ☐ No
5. START method of triage used?
☐ Yes ☐ No
6. Delays in patient transport?
☐ Yes ☐ No
7. Base Hospital Coordination adequate?
☐ Yes ☐ No
8. Were Cal-Fire Chiefs triage tags used?
☐ Yes ☐ No

Signature of Final Medical Group Supervisor: _____

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Patrick O. Duterte, Director

Steven P. Whiteley, M.D.
EMS Agency Medical Director

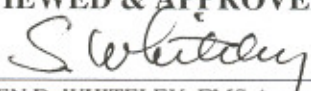
EMERGENCY MEDICAL SERVICES AGENCY
275 Beck Ave., MS 5-240
Fairfield, CA 94533
(707) 784-8155 FAX (707) 421-6682
www.solanocounty.com

Michael A. Frenn
Agency Administrator

POLICY MEMORANDUM #6190

October 21, 2004

REVIEWED & APPROVED BY:


STEVEN D. WHITELEY, EMS Agency Medical Director


MICHAEL A. FRENN, EMS Agency Administrator

SUBJECT: DUTY TO REPORT CHILD, ELDER AND DEPENDENT ADULT ABUSE

AUTHORITY: Welfare and Institutions Code §§ 15600-15601; California Health & Safety Code, Division 2.5 § 1797

PURPOSE/POLICY

To establish the parameters to recognize and report suspected child or elder and dependent adult abuse as mandated in the California Health and Safety Code and Institutions Code.

I. DUTY TO REPORT ELDER AND DEPENDENT ADULT ABUSE AS DEFINED

- A. **CHILD** means any person residing in this state 18 years of age and under.
- B. **ELDER** means any person residing in this state, 65 years of age or older.

- C. **DEPENDENT ADULT** means any person residing in this state, between the ages of 18 and 64 years, who has physical or mental limitations that restrict his or her ability to carry out normal activities or to protect his or her rights including, but not limited to, persons who have physical or developmental disabilities or whose physical or mental abilities have diminished because of age. A person who is temporarily disabled is considered a dependent adult.

II. ELDER & DEPENDENT ADULT ABUSE

- A. **Definition:** Abuse of an elder or dependent adult means either of the following:
- 1). Physical abuse (including sexual abuse), neglect (including self neglect), financial abuse, abandonment, isolation, abduction or other treatment resulting in physical harm or pain, or mental suffering.
 - 2). The deprivation by a care custodian of goods or services that are necessary to avoid physical harm or mental suffering.
- B. **Legal Requirements:** Pre-Hospital Care Personnel are mandated by Div. 2.5, Sect. 1797 of the California Health and Safety Code to report suspected abuse.
- C. **Required Actions for Elder and Dependent Abuse in a Person's Home:** In any situation in which Pre Hospital Care Personnel have reason to **suspect** abuse in a dependent adult or elderly person's own home:
1. The EMT/EMT-P will make reasonable efforts to transport a physically or sexually abused or neglected adult needing medical care to a receiving hospital for evaluation and will immediately notify the Receiving Hospital and document on the PCR their suspicions.
 2. If, for any reason, the EMT/EMT-P do not transport the adult and the adult is in immediate need of protection, they will immediately notify the appropriate police or sheriff department having jurisdiction and remain on scene until law enforcement personnel have arrived.
 3. All pertinent observations will be documented on the Prehospital Care Report Form (PCR).
 4. The receiving Emergency Department Staff is to be notified of the suspicions. The name of the nurse receiving the report and notification of suspicions should be documented on the PCR.
 5. The EMT/EMT-P should complete and submit the Report of Suspected Dependent Adult/Elder Abuse Form [Soc 341 (6/00)]. This report should be faxed to Adult Protective Services at (707) 435-2440. The original report should

be sent to Adult Protective Services, 275 Beck Ave., MS 5-110, Fairfield, CA 94533, within 24 hours. This action must be documented on the PCR.

6. If the suspected abuse happens at a Long Term Care Facility, follow the same procedures as above. The initial report should be faxed to Long-Term Care Ombudsman at (916) 376-8914. To make a verbal complaint, call (800) 644-4194, or 800-231-4024 after business hours. The original report should be sent to Adult Protective Services, 275 Beck Ave., MS 5-110, Fairfield, CA 94533, within 24 hours. This action must be documented on the PCR.

III. CHILD ABUSE

- A. **Definition** – Child Abuse is any act of omission or commission that endangers or impairs a child’s physical or emotional health and development. Child abuse includes physical abuse, physical neglect, sexual abuse and emotional maltreatment.
- B. **Required Actions for Child Abuse:**
 1. The EMT/EMT-P will make reasonable efforts to transport a physically or sexually abused, or neglected child needing medical care to a Receiving Hospital for evaluation, and will immediately notify and document their suspicions on the PCR.
 2. If for any reason the EMT/EMT-P do not transport the child and the child is in immediate need of protection, they will immediately notify the appropriate law enforcement agency and will remain at the scene until law enforcement personnel have arrived.
 3. The EMT/EMT-P will document all pertinent observations on the PCR.
 4. The receiving Emergency Department Staff is to be notified of the EMT/EMT-P’s suspicions. The name of the nurse receiving the report and notification of suspicions of abuse is to be documented on the PCR.
 5. The EMT/EMT-P should complete and submit the “Suspected Child Abuse Report (11166PC) form. This report is to be faxed to CPS, (707) 421-7535. The original report is to be sent to CPS, 275 Beck Ave., MS 5-230, Fairfield, CA 94533, within 24 hours. This action must be documented on the PCR.

